

Clinical Diagnosis & History:
 y/o female with approximately 2.6 cm invasive lobular carcinoma of right breast. Presents for TM, sentinel lymph node biopsy.

Specimens Submitted:

- 1: SP: Sentinel node #1, level one, right axilla
- 2: SP: Sentinel node #2, level one, right axilla
- 3: SP: Non-sentinel node #1 right axilla
- 4: SP: Sentinel node #3 level 1 right axilla
- 5: SP: Right breast

DIAGNOSIS:

- 1) LYMPH NODE, SENTINEL #1 LEVEL 1 RIGHT AXILLA; BIOPSY:
 - ONE BENIGN LYMPH NODE (0/1).
 - ADDITIONAL H&E STAINED SECTIONS AND IMMUNOHISTOCHEMICAL STAINS FOR CYTOKERATIN (AE1:AE3) SHOW NO EVIDENCE OF METASTATIC CARCINOMA.
- 2) LYMPH NODE, SENTINEL #2 LEVEL 1 RIGHT AXILLA; BIOPSY:
 - ONE BENIGN LYMPH NODE (0/1).
 - ADDITIONAL H&E STAINED SECTIONS AND IMMUNOHISTOCHEMICAL STAINS FOR CYTOKERATIN (AE1:AE3) SHOW NO EVIDENCE OF METASTATIC CARCINOMA.
- 3) LYMPH NODE, NON-SENTINEL #1 RIGHT AXILLA; BIOPSY:
 - ONE BENIGN LYMPH NODE (0/2).
- 4) LYMPH NODE, SENTINEL #3 LEVEL 1 RIGHT AXILLA; BIOPSY:
 - METASTATIC CARCINOMA IN THE FORM OF MICROSCOPIC CLUSTERS (A/R)25 CELLS) MEASURING <2MM (MICROMETASTASIS) IS IDENTIFIED ON H&E STAINED SECTIONS AND CYTOKERATIN IMMUNOHISTOCHEMICAL STAINS (1/1).
- 5) BREAST, RIGHT: TOTAL MASTECTOMY:
 - INVASIVE LOBULAR CARCINOMA, PLEOMORPHIC TYPE (E-CADHERIN NEGATIVE), MEASURING 2.5 CM IN LARGEST DIMENSION GROSSLY.
 - LOBULAR CARCINOMA IN SITU (LCIS) IS ALSO IDENTIFIED, PLEOMORPHIC TYPE.
 - THE INVASIVE CARCINOMA IS LOCATED IN THE UPPER OUTER QUADRANT.
 - NO INVOLVEMENT OF THE NIPPLE BY EITHER IN SITU OR INVASIVE CARCINOMA IS IDENTIFIED.
 - CALCIFICATIONS ARE PRESENT IN THE IN SITU CARCINOMA.
 - NO VASCULAR INVASION IS NOTED.

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1CD-0-3
 carcinoma, infiltrating pleomorphic (lobular type)
 Site: breast, nos C50.9 bx 10/22/14 803213

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HPV Discrepancy		
Prognostic Factor Discrepancy		
Quality of Pathology		
Case Review		
Reviewer Initials		

10/22/14

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- 2
- NO INVOLVEMENT OF THE SURGICAL MARGINS BY EITHER INVASIVE OR IN SITU CARCINOMA IS IDENTIFIED.
 - NO SKIN INVOLVEMENT BY CARCINOMA IS IDENTIFIED.
 - THE SKIN SHOWS SEBORRHEIC KERATOSIS.
 - THE NON-NEOPLASTIC BREAST TISSUE SHOWS FIBROCYSTIC CHANGES.
 - THE LYMPH NODE STATUS IS AS FOLLOWS (EXPRESSED AS THE NUMBER OF POSITIVE LYMPH NODES IN RELATION TO THE TOTAL NUMBER OF LYMPH NODES EXAMINED): LEVEL I: 0/3.
 - ER: 100% NUCLEAR STAINING WITH STRONG INTENSITY
 - PR: 30% NUCLEAR STAINING WITH WEAK TO MODERATE INTENSITY
 - HER2/NEU (HERCEPT): POSITIVE (STAINING INTENSITY OF 2+)

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

*** Report Electronically Signed Out ***

Special Studies:

Result

Special Stain

Comment

AE1:AE3

NEG CONT

IMM RECUT

AE1:AE3

AE1:AE3

NEG CONT

IMM RECUT

AE1:AE3

NEG CONT

IMM RECUT

ER-C

PR-C

HER2-C

E-CADHERIN

NEG CONT

NEG-HER2

IMM RECUT

Gross Description:

1). The specimen is received fresh for frozen section consultation, labeled "Sentinel node #1, level 1, right axilla" and consists of a lymph node measuring 0.4 x 0.3 x 0.3 cm. The lymph node is bisected and submitted for frozen section diagnosis.

Summary sections:

FSC - frozen section control

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2). The specimen is received fresh for frozen section consultation labeled, "Sentinel node #2, level 1, right axilla" and consists of a lymph node measuring 0.5 x 0.5 x 0.4 cm. The lymph node is bisected and submitted for frozen section diagnosis.

Summary sections:
FSC - frozen section control

3). The specimen is received in formalin, labeled "non-sentinel node number one right axilla" and consists of one single pink-tan firm lymph node measuring 0.3 x 0.3 x 0.2 cm. Entirely submitted in one cassette.

Summary of sections:
LN -- lymph nodes

4). The specimen is received in formalin, labeled "sentinel node #3 level 1 right axilla" and consists of one single lymph node measuring 0.5 x 0.4 x 0.3 cm. The lymph node is entirely submitted.

Summary of sections:
LN- lymph node

5) The specimen is received fresh, labeled "right breast, stitch marks axillary tissue level 1" and consists of a breast with attached axillary tail. The breast measures 15.2 x 10.8 x 2.1 cm with overlying skin ellipse measuring 14.1 x 6.0 x 8.0 cm. Situated on the skin surface is an everted nipple measuring 2.8 x 0.8 x 0.2 cm and areola measuring 2.8 x 2.7 cm. The skin shows no grossly visible scars. There is a brown plaque-like lesion superior-lateral to the nipple measuring 1.7 cm in diameter. A suture demarcates the axillary tail which measures 8.5 x 3.3 x 0.5 cm. The posterior surface of the breast is inked black and the specimen is serially sectioned to reveal an ovoid white-tan tumor located in the upper outer quadrant 2.5 x 1.6 x 1.3 cm, located 1.3 from the deep margin. The remaining breast tissue shows yellow lobulated adipose tissue admixed with white fibrous soft tissue with no other grossly identifiable lesions. The axillary tissue is dissected to reveal several possible lymph nodes, ranging in size from 0.3 cm to 0.9 cm. Representative sections of the mastectomy specimen and all identified axillary lymph nodes are submitted. Tissue submitted for TPS.

Summary of sections:
N - nipple
NB - nipple base
S - skin lesion
D - deep margin
T - tumor
UIQ - upper inner quadrant

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LIQ - lower inner quadrant
 UOQ - upper outer quadrant
 LOQ - lower outer quadrant
 LNS -- individual level 1 lymph nodes

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Summary of Sections:

Part 1: SP: Sentinel node #1, level one, right axilla

Block	Sect.	Site	PCs
1		FSC	1

Part 2: SP: Sentinel node #2, level one, right axilla

Block	Sect.	Site	PCs
1		fsc	1

Part 3: SP: Non-sentinel node #1 right axilla

Block	Sect.	Site	PCs
1		ln	1

Part 4: SP: Sentinel node #3 level 1 right axilla

Block	Sect.	Site	PCs
1		ln	1

Part 5: SP: Right breast

Block	Sect.	Site	PCs
1		D	1
2		LIQ	2
1		LNS	1
2		LOQ	2
1		N	1
1		NB	1
2		S	1
2		T	2
2		UIQ	2
2		UOQ	2

Intraoperative Consultation:

Note: The diagnoses given in this section pertain only to the tissue sample examined at the time of the intraoperative consultation.

1) FROZEN SECTION DIAGNOSIS: BENIGN.
 PERMANENT DIAGNOSIS: SAME

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2)

FROZEN SECTION DIAGNOSIS:

BENIGN. (RAS)

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PERMANENT DIAGNOSIS:

SAME

5/

** End of Report **